

--SUMMARY--

Decision No. 1805/18

11-Jul-2018

D.Revington - M.Christie - A.Signoroni

- Continuing entitlement

No Summary Available

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- WSIA

Other Case Reference

- [w3918n]

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1805/18

BEFORE:

D. Revington : Vice-Chair
M. Christie : Member Representative of Employers
A. Signoroni : Member Representative of Workers

HEARING:

June 14, 2018 at Toronto
Oral

DATE OF DECISION:

July 11, 2018

NEUTRAL CITATION:

2018 ONWSIAT 2285

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) decision dated
December 21, 2016

APPEARANCES:

For the worker:

C. Oliverio, Paralegal

For the employer:

T. P., Manager for the Employer

Interpreter:

Not applicable

**Workplace Safety and Insurance
Appeals Tribunal**

505 University Avenue 7th Floor
Toronto ON M5G 2P2

**Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail**

505, avenue University, 7^e étage
Toronto ON M5G 2P2

REASONS

(i) Introduction

- [1] The worker appeals a decision of the Workplace Safety and Insurance Board's (WSIB) (the Board) Appeals Resolution Officer (ARO) dated December 21, 2016. That decision found the worker had no ongoing entitlement for a low back and right hip strain, no entitlement to loss of earnings (LOE) benefits after March 10, 2015, and was not entitled to reimbursement for physiotherapy costs from July 30, 2015 to October 27, 2015. The worker appeals all three issues to the Tribunal.

(ii) Background

- [2] The worker was 32 years old when he fell and injured his back and buttocks on February 24, 2015. He claims his back pain got worse during the following few days, making him unable to do any work after March 10, 2015. Over the course of three years the worker has been examined by a variety of specialists, none of whom have made any significant organic findings. The worker continues to claim he has had severe disabling pain since the accident. The Board has allowed only initial entitlement for health care benefits for the acute phase of the injury.

(iii) Law and Board policy

- [3] The worker was injured in 2015; that means the *Workplace Safety and Insurance Act, 1997* (the WSIA) applies. The Tribunal has jurisdiction to hear this appeal pursuant to section 123 of the WSIA.
- [4] Section 126(1) of the WSIA requires the Tribunal to apply Board policies when making its decisions. The Board has identified Policy Packages #30, #95, #212, #224, #281 and #300, Revision #9 as applicable to this appeal. We will make specific reference to a few of these policies in the course of this decision.

(iv) Evidence

(a) The accident

- [5] The worker was a construction worker. He operated a telehandler, or "zoom-boom" machine. On February 24, 2015 he slipped and fell when he was shoveling snow on a flatbed trailer. He landed on his back and buttocks. He reported his accident to his foreman. Although the worker testified that he felt pain at the time of the injury, he did not feel the injury was significant enough to seek medical attention.
- [6] The worker was 32 years old at the time of the accident. Other than a workplace shoulder injury in 2009, for which the worker had not lost time from work, the worker testified that he was in good health and had no other injuries or medical conditions prior to the 2015 accident.
- [7] The worker testified that each day after the accident his pain became worse. He continued working full time for the next eight working days, until Friday, March 6, 2015. The worker testified that he thought that working in the telehandler machine made his back worse, because there was no suspension, no heat, and he had difficulty getting in and out of the machine.

- [8] The worker saw Dr. Praglowski, his family doctor, on Friday March 6, 2015. Dr. Praglowski completed a short report stating the worker would be off work from March 6 to 13, 2015 “for medical reasons.” The worker stated that after his doctor’s appointment he called his foreman, to advise him that his doctor had told him to stay off work. However, the worker testified that his foreman told him he had to come to work to complete some paperwork.

(b) Modified work

- [9] The worker came to work on Monday March 9, 2015. He testified that he sat on a broken chair, and it took him all day to fill out some forms.
- [10] Written documentation in the Case Record shows that while he was at work on March 9, 2015, the worker signed a letter acknowledging that the employer had offered him modified duties at no wage loss. The worker was actually performing modified duties on March 9, 2015, which the employer had provided, and for which the worker was paid full wages.
- [11] The worker stated he called the employer the following day, on March 10, 2015, and advised that he would not be coming to work because his doctor ordered him to stay home. There was no new medical opinion or report that suggested the worker’s condition was any worse than it had been the day before. On the same day, March 10, 2015, the employer couriered a letter to the worker. This letter confirmed that the same modified duties the worker was performing the previous day remained available at no wage loss. The modified duties were filing folders and documents, reviewing training materials, and any other duties within his restrictions that would be delegated by his foreman. The letter also stated that a chair would be provided for the worker, he could work at his own pace, take breaks and rest as needed, a taxi would take him to and from work, and the worker would be reasonably accommodated to take medical appointments.
- [12] When questioned, the worker acknowledged that he had never shown Dr. Praglowski this letter with the modified duties and accommodation the employer had offered.

(c) The worker’s medical reports after March 10, 2015

- [13] An x-ray of the worker’s low back taken on March 6, 2015, was essentially normal, except for a minor narrowing of the L5-S1 disc space. The worker began to see his family doctor frequently: from March 3, 2015 to July 28, 2015, he saw his doctor 20 times. He explained that this was because Dr. Praglowski was trying out different pain medications and muscle relaxants, some of which made him ill. On March 19, 2015, 10 days after the last day the worker had been at work, Dr. Praglowski recorded that the worker claimed to be in “horrible pain.”
- [14] Dr. Praglowski stated in a letter dated April 20, 2015 that the worker had persistent pain and she saw him weekly. She said the worker had tried physiotherapy, but it had not helped. She noted the worker had been on anti-inflammatories, muscle relaxants, and painkillers. In a Board Health Professional’s Report (Form 8) which Dr. Praglowski completed on April 21, 2015, she recorded the worker as rating his pain at 10/10.
- [15] Dr. Jhas, a neurosurgeon who examined the worker on August 5, 2015, was unable to identify any organic reason for the worker’s pain. Dr. Jhas found the worker had normal strength in his upper and lower extremities, a normal sensory examination, and normal reflexes. A CT scan of the worker’s back had been taken on May 6, 2015. Dr. Jhas noted the CT scan did not show any significant herniations or fractures.

(d) Perianal abscess and fistula

[16] In July 2015, Dr. Praglowski noted the worker had developed rectal pain and bleeding, caused by multiple anal abscesses. The worker was seen by Dr. Roth, a gastroenterologist. Dr. Roth noted the medications the worker was taking at this time included Percocet, Pregabalin and Baclofen. After an MRI taken on July 22, 2015 revealed a perianal fistula, Dr. Roth referred the worker to Dr. Heisler, a surgeon. Dr. Heisler performed a fistulotomy in January 2016.

[17] The worker testified that his back pain was not affected by the fistula. He alleged the fistula was caused by the work accident, but there is no medical opinion to support such a connection.

(e) Further medical reports

[18] Dr. Praglowski recorded in November 2015 that another MRI of the worker's lower spine showed normal results. Despite this, she reported that the worker stated that he was getting worse. She reported that the worker continued to suffer back pain, could not sit, and walked with a cane. She noted that he had tried many medical treatments for his back, but none of them worked.

[19] The Board had denied initial entitlement for the worker's claim. The worker applied for Ontario Disability Support Program (ODSP) benefits. On November 6, 2015, Dr. Praglowski completed an ODSP form to assist the worker with his application for benefits. Dr. Praglowski noted the worker had fallen at work, developed severe back pain, and "for some reason never got better." She reported that the worker was able to walk short distances with the help of a cane. She noted the worker was on a number of medications, including painkillers Tylenol 3, Percocet, and other muscle relaxants. She wrote "I can't really explain why he suffers so much."

[20] In February 2016, the Board granted initial entitlement for the worker's back claim. However, the Board allowed the worker's claim for health care benefits for only the acute injury. No entitlement was allowed for LOE benefits, because the Board found the employer had offered suitable work. The Board adjudicator's opinion was that the worker remained off work due to non-work-related conditions. Nevertheless, the worker was still reporting to the Board that he had severe back pain that was progressively getting worse.

[21] On May 12, 2016, the worker underwent another MRI of his back. Dr. J. Mamelak, a diagnostic radiologist, compared the results to an MRI taken on September 30, 2015. Dr. Mamelak reported that the disc spaces were normal, the facet joints were normal, the central canal and neural foramina were patent. He stated that at L5-L6 "Although the sacroiliac joints are not completely imaged, acute on chronic erosive changes are evident." Dr. Mamelak stated that this "may be a cause of back pain."

[22] On August 3, 2016, Dr. Silverberg, a specialist in internal medicine, examined the worker. He noted the worker claimed he had back pain ever since his accident in February 2015, that he had not improved since the accident, and was unable to return to his regular work because he would have to carry heavy items. Dr. Silverberg's opinion was that the worker's low back pain was due to soft tissue injuries to his lumbosacral and cervical spines from his February 2015 work injury. Dr. Silverberg did not anticipate that the worker would improve, or that the worker could return to a construction job.

(f) ARO decision

[23] On December 21, 2016, ARO Danos denied the worker's request for ongoing entitlement, LOE benefits, and reimbursement of physiotherapy costs beyond July 30, 2015. The ARO held the evidence did not establish a causal link between the minor low back and hip injury and the later development of symptoms making the worker unable to work.

[24] Following the ARO decision, the worker's representative submitted further information relating to the worker's medical condition for the purposes of the appeal to the Tribunal. This information included.

- A March 27, 2017 report from Dr. Praglowski, in which she states the worker "has had all kinds of investigation one can have for their lower back pain." Dr. Praglowski stated that the worker was referred to a pain clinic, he was now being prescribed more narcotics, but nothing improves his pain. "As a matter of fact, his pain remains as a sort of mystery to me."
- A June 19, 2017 decision of the Social Benefits Tribunal granted the worker ODSP benefits. This decision relied on Dr. Praglowski's report that the worker had suffered back pain since his work injury, and that he continued to get worse. The Social Benefits Tribunal adjudicator noted the worker was taking significant narcotic medication, which provided only minor relief of his pain. The adjudicator also found the worker's testimony about his limitations to be credible. The decision found that, based on a rating of the worker's Activities of Daily Living, the worker would have found it difficult to secure employment and to be considered a satisfactory employee in any workplace. The ODSP award is to be reviewed in three years, due to the applicant's young age.
- The worker was seen by Dr. Rod at the Toronto Poly Clinic, which specializes in treating pain management. A March 9, 2018 report from Dr. Rod solicited from the worker's representative, diagnosed the worker with chronic post-traumatic lower back pain, chronic post-traumatic erosive lumbar spine arthritis, and chronic right lumbar polyradiculopathy. In response to a written question from the worker's representative, Dr. Rod stated the worker "had no history of any back pain before his injury. The accident in question is the cause of his current condition."
- A May 15, 2018 report of Dr. Praglowski, solicited by the worker's representative, stated the worker's current diagnosis was non-specific musculoskeletal pain and the beginning of degenerative disc disease in the lumbar spine. She stated that in her opinion, while the worker's low back injury was complicated by anal fistula, the origin of his low back pain was the soft tissue injury to the lumbosacral spine. She stated the worker could not return to construction work, but since he was only 34, she did not believe he was completely disabled. She conceded that "all investigations failed to reveal any permanent damage, fracture, spine injury, etc."

(g) Additional testimony

[25] At the hearing, the worker stated his back pain is still severe. He said the pain is so bad at night it is difficult to breathe. He uses a cane and a back brace. He stated that if he washes dishes he has to lie down because of pain. He continues to see his family doctor once a month. The worker has rejected injection treatment and surgery.

[26] The worker testified that, in addition to his other medications, the Poly Clinic has prescribed medical marijuana, which he finds calming. He stated that since the accident he has not thought about retraining, he has focused on getting better, but he realizes he may need to be retrained. The worker stated that he spends his days sitting at home, smoking marijuana and thinking that he is in a bad spot and needs to do something.

(h) Submissions

[27] In her detailed submissions, the worker's representative argued that the Board erred initially in thinking the accident was not severe. She cited Dr. Mamelak's comment on the MRI about erosive changes which could be a cause of back pain. She argued the worker had no pre-existing or co-existing conditions. She noted that Dr. Silverberg, Dr. Rod, and Dr. Praglowski had all stated that the worker's pain is a result of the accident. She suggested the Maximum Medical Recovery date would be the date of Dr. Silverberg's report of August 2016.

[28] The worker's representative also cited *Decision No. 2120/14*, where a worker's appeal was granted on the basis of permanent aggravation of a pre-existing degenerative condition.

[29] The employer's representative supported the Board's decision. He stated the employer had offered modified work to the worker. He noted that there was no definitive diagnosis of the worker's condition, and no objective evidence to support the disability. When asked if the modified work offered would have been sustainable, he stated that the offer of modified work was like a living document, which we understand to mean the employer could have made further offers of modified work depending on the evolution of the worker's medical restrictions. He noted that he was not with the company at the time of the events in question, but that there are a number of instances where permanent modified duties have been made available to injured workers.

(v) Analysis

(a) Ongoing entitlement for a low back and right hip strain

[30] Many Tribunal decisions have characterized the test for ongoing entitlement to be whether the worker's compensable injury was a "significant contributing factor" in his or her ongoing condition (see *Decision Nos. 3407/17, 770/15*). In the context of this case, that means the issue is whether the worker's injury was a significant contributing factor in the worker's organic condition after March 9, 2015.

[31] We find the worker does not have continuing entitlement for organic low back and hip pain caused by the injury. While the worker has had ongoing pain which he alleges was caused by the injury, we find that there is no organic basis for these complaints beyond an initial six-week period.

[32] First, we find the slip and fall accident would not be expected to have caused an injury that would render this otherwise healthy 32-year-old worker unable to work for over three years. As a Board adjudicator noted in a decision of February 18, 2016, the worker would have been expected to recover from this injury within six to eight weeks. Dr. Praglowski initially authorized the worker to be off work for only seven days. The back injury was not significant enough to initially cause the worker to seek medical treatment. Yet, the worker reported his pain increased over the next few days to extreme levels. Even after he stopped working, the worker alleged he

continued to get worse, with pain consistently rated as 10/10, and for which significant narcotic medications provided only minor relief.

[33] Second, the degree of pain is not consistent with the organic findings. There has been no reasonable medical explanation for how, on an organic basis, the worker's intense and protracted pain could have been caused by the accident.

[34] The radiological evidence provides scant support for a relationship between the accident and the worker's back pain. The fall did not result in any fracture or obvious damage to the lumbar spine. The worker had an x-ray of his low back on March 6, 2015, which did not show any damage. Dr. Jhas noted that a May 6, 2015 CT scan did not show any significant herniations or fractures. A November 5, 2015 MRI of the worker's low back showed normal results. A May 12, 2016 MRI was also entirely normal, except that Dr. Mamelak stated that "Although the sacroiliac joints are not completely imaged, acute on chronic erosive changes are evident" and that this "may be a cause of back pain."

[35] It is important to note that Dr. Mamelak did not link the chronic erosive changes, or the worker's pain, to the slip and fall accident in February of the year before. He does not state that the worker's accident could have caused degenerative changes over a year later.

[36] A review of the Tribunal's Medical Discussion Paper on Low Back Pain suggests there would likely be no organic relationship between the accident and the worker's complaints.

[37] The Tribunal provided a copy of its Medical Discussion Paper on Low Back Pain to the parties prior to the hearing.¹ The Low Back Pain paper states that in common back strain, injured soft tissues become painful. It is not possible to determine precisely which structure is affected. Most back strains heal in a few days or at most a few weeks, and are not likely to be a source of continuing pain.

[38] There does not appear to be any organic reason why the worker's soft tissue injury would have continued to cause pain after a few weeks. The Discussion Paper also states that back pain may be linked to compression of a spinal nerve root, but there was very little radiological evidence of this worker having nerve root compression.

[39] Further, the fact this now 35-year-old worker has degenerative changes in his spine does not necessarily support a relationship to back pain from a slip and fall, and certainly not back pain as extreme as the worker claims to be experiencing. As the Discussion Paper states, degenerative disc disease would be better expressed as normal age or genetic-related changes, and that "injury does not cause aging changes."

¹ As the Ontario Divisional Court has recognized (*Kamara v Workplace Safety and Insurance Appeals Tribunal*, 2009 CanLII 26353 (ON SCDC) at para 4), the Tribunal commissions Medical Discussion Papers to provide general information on medical issues which may arise in Tribunal appeals. Each Medical Discussion Paper is written by a recognized expert in the field. Each expert writes the paper so that it can be understood by lay people.

Recognizing that medical issues may be controversial, the Tribunal requests each expert to write the Medical Discussion Paper so that it presents a balanced view of the current medical knowledge on the topic.

A Medical Discussion Paper does not necessarily represent the views of the Tribunal. A Vice-Chair or Panel may consider the medical information provided in a Discussion Paper in an individual appeal, and may choose to rely on it, but the Tribunal is not bound by a Medical Discussion Paper in any particular case. It is always open to the parties to an appeal to rely on a Medical Discussion Paper, or to distinguish or challenge it with alternative evidence.

[40] Third, although Dr. Praglowski, Dr. Silverberg, and Dr. Rod have all stated that the accident has caused the worker's pain, we find that a close reading of each of their medical opinions does not provide a reasonable organic link between the accident and the pain. As the employer's representative stated, there is no definitive diagnosis, at least from an organic perspective.

(b) Dr. Praglowski

[41] It is clear from Dr. Praglowski's comments such as - "I can't really explain why he suffers so much;" he "has had all kinds of investigations one can have for their lower back pain," and "his pain remains as a sort of mystery to me"- that she was perplexed by the cause of the worker's complaints.

[42] Dr. Praglowski's May 15, 2018 report, solicited by the worker's representative for the purpose of this appeal, stated the worker's current diagnosis was non-specific musculoskeletal pain and the beginning of degenerative disc disease in the lumbar spine. She stated that, in her opinion, while the worker's low back injury was complicated by anal fistula, the origin of his low back pain was the soft tissue injury to the lumbosacral spine. She conceded that "all investigations failed to reveal any permanent damage, fracture, spine injury, etc."

[43] Dr. Praglowski's link of the worker's pain to his soft tissue work accident was made after Dr. Rod and Dr. Silverberg had done so, and her report appears to follow those opinions. While the worker undoubtedly did suffer back pain from the soft tissue injury, Dr. Praglowski does not offer an explanation for how, organically, the worker's pain could have increased and persisted in the severe manner the worker claimed it had for over three years after he stopped working. The comments she made on numerous occasions strongly supports that she was unable to discern an organic cause for the worker's back pain.

(c) Dr. Rod

[44] Dr. Rod's March 9, 2018 opinion, which was also solicited by the worker's representative for this appeal, essentially concludes the accident was the cause of the injury because the worker did not have back pain before the injury. Dr. Rod does not provide support for how the worker's ongoing pain is consistent with organic causes from an injury three years before, which is the question before us. He does not explain how he relates his diagnosis of erosive lumbar spine arthritis to the accident, or how he relates his diagnosis of radiculopathy to any nerve root. As the Medical Discussion Paper on Low Back Pain states "nerve root or radicular pain (i.e., neuropathic pain) is different from and must be distinguished from the local or "referred" pain from low back muscles, ligaments, annulus, periosteum, and facet joints. It is caused by very specific pathology, e.g., nerve root compression from a disc fragment that also causes chemical irritation between the fragment and nerve root – this results in neurogenic pain."

(d) Dr. Silverberg

[45] Dr. Silverberg's reasoning process was similar. Since the worker's back pain started after the accident, Dr. Silverberg concluded it was caused by the "soft tissue injuries to the lumbosacral and cervical spines that he suffered in a work injury in February 2015 and he continues to experience pain from that injury with nerve root symptoms down the right lower extremity."

[46] Dr. Silverberg's opinion relied upon incorrect information. First, there was no evidence of injury to the worker's cervical spine in the worker's fall of February 24, 2015. Since

Dr. Silverberg was under the mistaken impression there was a cervical injury, this suggests his opinion is seriously flawed. Second, Dr. Silverberg does not explain what radiological evidence supports nerve root impairment to the worker's lumbosacral spine, or how a soft tissue injury could result in nerve root symptoms. We find it significant that Dr. Jhas examined the worker a year before Dr. Silverberg, and found no sensory impairment.

(e) Fistula

[47] Since we have found there was no organic entitlement after April 7, 2015 for the worker's injury, and as the perianal abscesses developed in approximately July of 2015, we make no finding on what impact that condition had on the worker's back condition.

(f) Decision No. 2120/14

[48] The worker's representative cited *Decision No. 2120/14*. In that case, the Vice-Chair found a workplace accident had caused a permanent aggravation of the worker's pre-existing asymptomatic degenerative conditions in the neck, low back, and right shoulder.

[49] *Decision No. 2120/14* is distinguishable from the present case, for two reasons. First, the medical imaging in *Decision No. 2120/14* demonstrated the existence of pre-existing mild to moderate degenerative conditions and spinal stenosis at the time of the work injury. In the case before us, there was little in the way of medical imaging showing a pre-existing degenerative condition that could have been aggravated at the time of the injury, and no spinal stenosis. Second, unlike *Decision No. 2120/14*, the medical reports do not support that the accident had permanently aggravated the worker's underlying asymptomatic degenerative conditions.

(g) Our conclusion on this issue

[50] We have assessed and weighed the evidence in this case. We recognize that there is evidence on both sides of this issue, including medical opinion which supports the worker's position. We also acknowledge the worker appears to sincerely believe he is disabled by pain. However, we must make our determination on a review of the entire evidence in view of the law and policy. We have found the worker's ongoing pain is not consistent with the organic injury which he suffered on February 24, 2015. The nature of the slip and fall accident would not be expected to have resulted in a significant injury, the complaints of pain are not consistent with the injury, and the medical opinion does not provide a reasonable link between the accident and the extreme pain and disability the worker states he is experiencing. We find on the balance of probabilities that the worker's accident did not make a significant contribution to the worker's ongoing organic disability.

[51] We agree with the view of the Board adjudicator, and accept the opinion expressed in the Medical Discussion Paper, and find that the worker was not likely to have been disabled from organic causes from the injury for longer than six weeks. Six weeks from February 24, 2015, would be April 7, 2015. We find the worker does not have ongoing organic entitlement after April 7, 2015.

[52] We stress that the issue of entitlement on the basis on a non-organic injury has not been ruled on by the Board, it was not argued before us, and we did not have jurisdiction to consider it.

(h) Entitlement to LOE benefits from March 10, 2015

- [53] A worker is entitled to LOE benefits if the loss of earnings is a result of a compensable injury (WSIA section 43(1)). The loss of earnings must be *a result of the injury*.
- [54] Not accepting suitable and available employment after an injury (section 43(2)(b)) means the loss of earnings is not a result of the injury; it is a result of the worker not accepting the suitable and available employment. As the Divisional Court has recognized, there is longstanding Tribunal jurisprudence holding that a refusal of suitable work is a ground for ceasing LOE benefits (*Outar v. Workplace Safety and Insurance Appeals Tribunal* (2013) ONSC 1697 (CanLII) at para 5).
- [55] When a worker has been injured at work, sections 40(1) and (2) of the WSIA establishes the worker and employer have a duty to co-operate in the worker's early and safe return to work. Suitable work is defined in Board *Operational Policy Manual* (OPM) Document No. 19-02-01 as "post-injury work (including the worker's pre-injury job) that is **safe, productive, consistent with the worker's functional abilities**, and that, to the extent possible, **restores the worker's pre-injury earnings**." [emphasis in original]
- [56] We agree with the Board that the worker is not entitled to LOE benefits because the employer had offered suitable modified work at no wage loss.
- [57] The worker returned to work on March 9, 2015 and was able to perform the duties provided to him. The worker signed a form acknowledging the employer had offered him modified duties at no wage loss. The following day the employer sent the worker a letter offering the same duties that the worker was able to perform the previous day.
- [58] The modified duties were filing folders and documents, reviewing training materials, and any other duties within his restrictions that would be delegated by his foreman. The employer's letter also stated that a chair would be provided for the worker, he could work at his own pace, take breaks and rest as needed, a taxi would take him to and from work, and the worker would be reasonably accommodated to take medical appointments. We find these duties were within the worker's medical restrictions, and were suitable and available. Strong support for this view is provided by the fact the worker was able to perform the modified duties on March 9, 2015.
- [59] The worker told the employer on March 10, 2015, that his doctor had ordered him to stay home. Yet, the worker acknowledged that he had never told Dr. Praglowski about the modified duties and accommodation the employer had offered. We find it unlikely that Dr. Praglowski would have advised the worker that he could not have performed the modified work which was offered.
- [60] We recognize, however, that the modified work which the employer offered – filing documents, reading manuals, and other work as assigned – was not sustainable for an indefinite period of time. Tribunal jurisprudence has found that sustainability is an important factor in considering the suitability of modified work (see *Decision No. 2241/13*).
- [61] At this point there is no conclusive way to determine how long this modified work would have been available if the worker had accepted it. The employer's representative acknowledged the modified work that was offered to the worker was not intended to have been available for a lengthy period, although he also noted that the employer has been able to arrange for permanent accommodation for jobs on construction sites. Since the worker did not believe he was able to

perform any work, the employer did not make further offers of modified work after the worker rejected the offer made in March 2015.

[62] We have found that the worker would not likely have been disabled by organic causes for longer than six weeks from the February 24, 2015, accident. Six weeks from February 24, 2015, would be approximately April 7, 2015. The worker did not lose time from work for the first two weeks after the accident. The worker declined suitable modified work at no wage loss on March 10, 2015. That means the period of time in issue for the offer of modified work would have been from March 10, 2015, to April 7, 2015, which is a period of four weeks. We find that the modified work was likely sustainable for that four-week period, so that consequently the worker would not have suffered a wage loss during the period from the date of accident until April 7, 2015. Subsequent to April 7, 2015, we find the worker would not have been disabled for organic reasons related to the accident.

(i) Entitlement to reimbursement for physiotherapy costs from July 30, 2015 to October 27, 2015

[63] An injured worker's entitlement to health care is governed by sections 33(1), (2) and (7) of the WSIA and Board OPM Document No. 17-01-02. These provide that an injured worker is entitled to, and the Board shall pay for, such health care as may be necessary, appropriate and sufficient as a result of the injury.

[64] The Board denied the worker's requests for reimbursement for physiotherapy treatments taken between July 30, 2015, and October 27, 2015, on the grounds there was no entitlement for health care beyond the acute phase of the injury.

[65] As we have found the worker has no ongoing organic entitlement for the accident beyond April 7, 2015, we also find he is not entitled to reimbursement for physiotherapy costs from July 30, 2015, to October 27, 2015.

DISPOSITION

[66] The appeal is denied.

DATED: July 11, 2018

SIGNED: D. Revington, M. Christie, A. Signoroni